

PAMOL 650 TABLET

Each tablet contains: Paracetamol.....650 mg

This preparation contains PARACETAMOL. Do not take any other medicines containing PARACETAMOL at the same time.

Product Description:

Oblong, convex, white, plain tablet with single score on one side only.

Pharmacodynamics:

Paracetamol produces anti-pyresis through action on the hypothalamic heat-regulating center and analgesia by elevation of the pain threshold.

Pharmacokinetics:

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 30 minutes to 2 hours after ingestion. It is metabolised in the liver and excreted in the urine as the glucuronide and sulphate conjugates. Less than 5% is excreted as unchanged Paracetamol. The elimination half life varies from about 1 to 4 hours. Plasma protein binding is negligible at usual therapeutic concentrations but increases with increasing concentrations. It is reported that the binding of paracetamol to plasma protein did not occur with plasma concentration of less than 60mg per ml, corresponding to the usual therapeutic concentration. After toxic doses of paracetamol, up to 43% could be bound to plasma proteins.

Indication:

It is indicated as an analgesic and anti-pyretic in conditions accompanied by discomfort and fever such as the common cold and viral infections. It also provides effective analgesia in conditions involving musculoskeletal pain as well as in other painful disorders such as headaches, dysmenorrhoea, myalgia, and neuralgia.

Recommended Dosage:

The adult dose is 650mg (1 tablet) to 975 mg (1½ tablets), 3 or 4 times daily. Not more than 3.9g (6 tablets) in 1 day unless advised by the physician.

Route of Administration:

Oral

Contraindication:

It is contraindicated in patients who are hypersensitive to it and also in patients who have liver impairment.

Warnings and Precautions:

Paracetamol should be given with care to patients with impaired kidney or liver function. Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Interactions with Other Medicaments:

Paracetamol should also be given with care to patients taking other drugs that affect the liver.

Paracetamol hepatotoxicity has been reported to be enhanced by phenobarbitone, alcohol and doxorubicin.

Contradictory pharmacokinetic results have been reported in patients receiving both chloramphenicol and paracetamol, ranging from increase to decrease and to no significant change in chloramphenicol half-life. A combination of paracetamol and codeine has also enhanced warfarin anticoagulation activity. Paracetamol has been reported to increase zidovudine's haematological toxicity.

Pregnancy and Lactation:

As there are no adequate and well-controlled studies in pregnant woman, this drug should be used only if clearly needed. The use of this drug in late pregnancy is not recommended because of the effects on the foetal cardiovascular system.

Since safety of the use of this preparation in pregnancy, during lactation, or in the childbearing age has not been established, use of the drug in such patients requires that the potential benefits of the drug be weighed against its possible hazard to the mother and child.

Side Effects:

Side effects of paracetamol are usually mild, though haematological reactions such as agranulocytosis, thrombocytopenia and pancytopenia have been reported. Other allergic reactions occur occasionally. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

Symptoms and Treatment of Overdose:

Symptoms following a potentially hepatotoxic overdose may include nausea, vomiting, diaphoresis and general malaise. In paracetamol overdosage, regardless of the quantity of paracetamol ingested, acetylcysteine should be administered immediately if 24 hours or less have elapsed from the reported time of ingestion. Acetylcysteine is given by intravenous infusion in an initial dose of 150mg per kg body weight over 15 minutes followed by 50mg per kg body weight over 4 hours and then 100mg per kg over the next 16 hours. Alternatively, methionine 2.5g may be given by mouth every 4 hours to a total of 4 doses. The following additional procedures are recommended. The stomach should be emptied promptly by lavage or by induction of emesis with syrup of ipecac. A serum paracetamol assay should be obtained as early as possible, but no sooner than four hours following ingestion. Liver function studies should be obtained initially and repeated at 24 hours intervals.

Effects on Ability to Drive and Use Machine: Not applicable.

Storage Conditions:

Store at or below 30°C.

Shelf life:

3 years

Pack Size:

Blister pack: A box of 3 X 10, 10 x 10, 50 x 10 and 100 x 10 tablets per strip. Bottle pack : A bottle of 180 tablets.

Pack Size (export only): A bottle of 1000 tablets.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:

Sunward Pharmaceutical Sdn. Bhd.

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